CONSULT FOR AFRICA

The Marketing Operating Brief

The agency, the four brands, and how we scale

Prepared for Toluwanimi Olayeni, Marketing and Communications Lead
Consult for Africa internal. Not for circulation outside the firm.
17 September 2026

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Lagos and Abuja, Nigeria

A letter from Debo

Nimi,

This is the brief I owe you. You have been running comms for us for months, you built and ran the Founding Circle push yourself, and you have been doing it without a written statement of what the function is responsible for or what I will judge it on. That is my omission and this document closes it.

I want to start with the thing that is not obvious from the way we have been working. You are running two businesses, not one, and the second one is the more important of the two.

The first is our own marketing. Four brands, one firm, and the job of turning what we know into demand for what we sell. That is real work and there is a great deal of it undone.

The second is the agency. Healthcare marketing is a published service line with its own page, its own capability set and its own engagement models, and it is the only service line we sell that is delivered entirely by your team. Every other line we sell draws on consultants. This one draws on you. That means the agency's capacity is your capacity, its quality is your quality, and its commercial performance is a line I will read every month alongside the consulting book.

What I want from that agency is specific, so I will say it plainly. I want us to build a world class healthcare marketing agency for hospitals. Not a general agency that takes health clients when they come, but a firm that hospital owners and group chief executives across this continent come to precisely because we understand how a hospital actually earns, which is the thing every general agency gets wrong when it sells them impressions instead of admissions.

We are well placed to do it for a reason that is worth being explicit about. Most agencies selling to hospitals have never run one. We have. When we tell a medical director that his problem is referral pipeline rather than reach, or that his tariff is the reason his marketing is not converting, that is not a positioning line, it is what our consulting work keeps finding. The agency's differentiator is the firm behind it, and your job is to make that legible in everything the agency produces.

Which brings me to the ask I care most about, and it is the reason I wanted you to have this document rather than the handbook Ejiro has.

I want you to plan now for scale and for execution efficiency, before the external clients arrive rather than after. The thing that will break as we take on outside accounts is not demand, because I can bring demand, and it is not talent, because we can hire. It is delivery. It is the point at which the same small team is running four brands of our own, servicing six paying hospitals, and discovering that everything we produce is bespoke, nothing is reusable, no two accounts are run the same way, and the only person who knows how any of it works is you. I have watched that happen to good agencies and it is what caps them at a handful of clients and a founder who cannot take a week off.

So I would rather we spent the next quarter productising than selling. Standard engagement shapes with known effort. A reusable asset library. One intake process and one review process that work the same way for our brands and for a client's. A capacity model that tells you, before you accept an account, whether you can actually deliver it. That work is unglamorous and it is the difference between an agency that reaches five clients and one that reaches fifty.

Ejiro joins you and has a separate handbook covering the firm, the brands, the positioning rules and the case study library. Read it, because you are the person who will hold her to it. Her document tells her what the rules are. Yours tells you that you own them.

On how we will work together. You own the function, the calendar, the standards and the team. I own the client relationships, anything touching a named client, the pricing, and any decision that commits the firm.

Where a piece of content is written in my name or touches a live engagement, it comes to me before it goes out, and I would rather see ten things I wave through than miss one I should have stopped.

The natural first step is for you to come back to me with the agency plan: what we productise, what we charge, who we hire and in what order, and what you need from me to make it possible.

Warm regards,

Dr Debo Odulana

Founding Partner, Consult for Africa

How this brief is organised

Part	What it covers
1	The two businesses you run, and the ambition
2	The firm, in the detail you need to sell it
3	The four brands, and the state each is actually in
4	The agency: responsibilities, service levels and KPIs
5	Scale and execution efficiency, which is the standing ask
6	The editorial engine and the clearance gate you operate
7	Running the team, including what Ejiro owns
8	The first hundred days at function level
9	Decisions needed from Debo
10	Appendices

Ejiro's document is `docs/marketing/cfa-marketing-handbook.pdf`. It is the reference layer: brand guides in full, the ten case studies written out, the glossary, the boilerplate. This brief does not repeat it. Where you need the long form, go there.

A note on targets. Every number in Part Four is a proposal, not a decision. They are set at levels that are defensible and achievable rather than aspirational, and they need Debo's sign off before they become the thing the function is measured on. Bring them to him as a set rather than one at a time, because they trade against each other.

Part One: The two businesses you run

1.1 Business one, our own marketing

Four brands, one firm. Consult for Africa itself, plus CadreHealth, Maarova and Mezo Health. The job is to convert what this firm knows into demand for what it sells, and the raw material is unusually good and almost entirely unused.

We have traced ten surgical cases end to end through a private clinic to find where the money leaked. We have recovered a building's true geometry from a CAD file and found the client's brief wrong by more than two hundred square metres. We have told a board that its revenue record was a nursing handover sheet. Almost none of that has ever been published, and no competitor in this market has anything comparable to

publish.

1.2 Business two, the agency

Healthcare marketing is one of the seven published service lines on consultforafrica.com, positioned as **empowering health brands through strategic marketing**, and sold to hospitals, health plans, non governmental organisations, life sciences, healthtech and diagnostics.

It is the only service line delivered entirely by your team rather than by consultants. That has three consequences worth stating up front. Its capacity is your capacity. Its margin is your team's utilisation. And its quality is your quality, personally, in a way that is not true of any other line.

What the agency sells today, as published. Eight capability areas.

Capability	What sits under it
Brand strategy and positioning	Brand architecture, value proposition, patient and persona segmentation, naming, identity, brand guidelines and governance
Market research and insights	Epidemiological and competitive analysis, patient journey mapping, qualitative focus groups, digital listening, health equity audits
Campaign planning and management	Multichannel media planning across digital and traditional, patient recruitment campaigns, advocacy programme management
Creative and content	Medical and consumer copywriting, graphic design and infographics, video and animation, medically vetted patient stories
Digital marketing and technology	Website and application development, social strategy and execution, email, SMS and telehealth platform marketing
Influencer and community engagement	Healthcare professional influencer programmes, patient advocate networks, community management
Public relations and advertising	Press relations and health journalism outreach, media training for spokespeople, print and outdoor, broadcast production
Execution, analytics and optimisation	Campaign dashboards, patient engagement analytics, customer relationship management and lead nurturing, testing and return on investment analysis

Three engagement models, also published.

Model	Shape
Retainer	Ongoing strategic support, digital roadmaps and quarterly campaigns, on a set monthly fee
Project	Fixed price for a one off initiative: a brand launch, a single campaign, a website build
Hybrid	A retained strategic core with project work layered on top, structured per client and per initiative

Five published differentiators. Pan-African perspective with local cultural and language nuance. Healthcare domain expertise, so we meet sector standards from day one. Strategy, creative, technology and analytics under one roof rather than four vendors to coordinate. We operate as an extension of the client's team through workshops, co-creation and deliberate knowledge transfer. And success measured in health outcomes rather than impressions.

That last one is the promise the KPI framework in Part Four has to make real, because at the moment it is a claim on a web page and nothing in our reporting proves it.

1.3 The relationship between the two, honestly

Running both with one team helps in three ways and hurts in two, and you should manage both sides deliberately.

It helps because our own brands are the proving ground. Everything we learn building CadreHealth's acquisition engine is directly transferable to a hospital trying to fill a service line. It helps because our own case studies are the agency's credentials. And it helps because the internal work smooths the gaps between external accounts, which is the thing that kills small agencies.

It hurts because internal work has no invoice attached and therefore always loses to client work when the week gets tight, which means our own brands quietly starve. And it hurts because an hour spent on our own marketing is an hour of capacity that was not sold, so the agency's apparent margin will look better than it is unless internal time is tracked as deliberately as billable time.

The discipline that fixes both: track internal brands as accounts. Give each of the four a notional retainer, a named owner, a calendar and a monthly report, exactly as though it were paying. You will immediately see when internal work is being eaten, and you will have honest utilisation numbers.

1.4 The ambition, stated

A world class healthcare marketing agency for hospitals, in Africa, built on the fact that the firm behind it has run hospitals.

The positioning that follows from it, and which should become the agency's own line once Debo has cleared the wording:

> Most agencies sell hospitals impressions. We have run hospitals, so we sell them admissions, referrals and the service lines that actually carry margin.

The proof that makes it credible is not a portfolio of pretty work. It is that we can look at a hospital's tariff, its referral pattern, its service mix and its occupancy and tell the owner which of those marketing can move and which it cannot. No general agency in this market can do that.

Part Two: The firm, in the detail you need to sell it

You already know most of this. It is here because the agency's pitch depends on it and because you will be asked.

2.1 What we sell

Consult for Africa is a healthcare management consulting firm founded in 2024, operating from Lagos and Abuja across Nigeria and the wider continent. Seven published service lines: clinical governance, strategy and growth, fractional leadership, digital health, health systems strengthening, diaspora expertise, and healthcare marketing.

In practice the work is bought as organisational audits, turnaround and growth mandates, board governance, venture structuring, and build and operate mandates where we set a facility up and then manage it.

The one line: we do not hand you a report and walk away.

2.2 How an engagement actually arrives

Almost entirely by referral and by Debo's own network, which is the single most important fact about our demand and the one that should shape your expectations of what marketing does here.

Marketing's job in a referral led business is not to generate cold leads. It is to do four things. Make the firm findable when somebody who has been told about us looks us up. Give the person who refers us something to send. Shorten the time between first conversation and signed engagement by having the proof already published. And build the standing that makes a hospital owner take the call in the first place.

Judge the function on that, not on inbound volume, and make sure Debo is judging it on that too.

2.3 The five stage progression

Options note for a prospect, then diagnostic, then strategy or plan, then delivery, then operate. Not every engagement goes the whole way and several have. The content that matters most sits at stage one and stage two, because that is where a prospect is deciding whether we are the firm that understands their problem.

2.4 How we price, and what it means for the agency

The consulting side prices resourced days at published grades, shows one visible concession, and anchors above the walk away rather than opening at the floor. Never put a price in marketing copy and never defend a price nobody has challenged.

For the agency the same philosophy applies and Part Five sets out what it means in practice: a published rate card logic, retainer floors, and scope defined tightly enough that a change is visibly a change.

2.5 The people

Dr Debo Odulana, Founding Partner. Physician, Sharjah, with a masters in international health management from Imperial College Business School. Former Chief Executive of Cedarcrest Hospitals Abuja, former Chief Innovation and Strategy Officer at Evercare Hospital Lekki, founder of Doctoora which he built to twenty one states and sold to Evercare. President of the Doctors Foundation for Care. He is a doctor and is Dr Debo Odulana in any formal context.

Tito Ipinmoye, Director of Product and Strategy. Runs day to day execution on audit engagements.

Dorothy, Director of Business Development and Growth.

You, leading marketing and communications. Ejiro joining your team.

Part Three: The four brands, and the state each is in

Full positioning, palettes and voice rules are in Ejiro's handbook. What follows is what you need as the person allocating effort: what each brand is for, what is actually true today, and what would move it.

3.1 Brand architecture

Consult for Africa is the parent and the credibility source. Maarova, CadreHealth and Mezo Health are products of the firm, living on CFA infrastructure today and each built to be able to leave later.

	Maarova	CadreHealth	Mezo Health
Sells to	Institutions	Individual professionals	Individual doctors, and facilities
The job	Know and grow your leaders	Own your career	A private practice without building one
Voice	Calm, evidence led	Peer to peer	Aspirational, respectful of seniority
Primary channel	LinkedIn and direct	X, Instagram, WhatsApp	Direct and referral

3.2 Consult for Africa

What would move it. Published proof. There is a library of ten anonymised case studies written up in Ejiro's handbook and not one of them has been published. Three insights articles and a LinkedIn cadence drawn from them would change how the firm reads to anybody who looks us up, which in a referral led business is the whole job.

The second thing that would move it is named references. We have exactly one cleared: the firm is the management partner for a twenty one bed drug and addiction rehabilitation centre in Lekki, and it has already run in our own advertising. Getting two or three more cleared is the highest value business development task available to marketing and the moment to ask is when an engagement is going well, not months later.

State. Wordmark in use, PNG only, no SVG master. Typography unresolved. Insights section exists and is thin.

3.3 CadreHealth

What it is. A career intelligence platform for Nigerian healthcare professionals across all sixteen cadres. Real salary data, honest hospital reviews, verified credentials, and a readiness score calibrated to UK, US, Canada and Gulf markets. Tagline: your career, your call.

The state, which decides every campaign you plan. Ten thousand two hundred and twenty six professional records are held. Six hundred and sixty have ever logged in. Ninety seven have verified an email. The rest is a cold import carrying a hard bounce rate of eleven per cent across roughly nineteen thousand sends, on which the provider's bounce webhook had never been configured, so nothing was ever suppressed. The imported name fields are transposed, so around twenty eight per cent of naive "Dr surname" greetings are wrong.

What this means for you as the lead. Do not let anybody, including Debo, plan a campaign against "the CadreHealth list" as though it were one audience. It is six hundred and sixty warm users and nine and a half thousand cold records with a deliverability problem attached. **No further bulk send should go out until the suppression back fill is done**, because another send at that bounce rate risks the sending domain, and a damaged domain takes months to repair and would affect every brand we run.

One person has ever paid for the subscription product. He is a diaspora paediatrician who converted himself off a cold list, which makes him the single best piece of evidence we have about what a paying member wants, and the relationship is currently in deficit because he was promised deliverables he never received.

Biggest open brand task in the firm: CadreHealth has no logomark and renders as type on CFA navy and gold.

3.4 Maarova

What it is. A psychometric assessment and coaching platform for healthcare leaders, calibrated to the African context rather than translated into it. Six modules, three streams, certified African coaches.

What you have already proved with it. The Founding Circle campaign is the template for how this brand acquires: a founder voice anchor post, a warm WhatsApp layer, a personal email in a colleague to colleague register, and a scarcity nudge before close, with every application reviewed personally. The hand picked clinician outreach worked because it was plain, personal, senior appropriate and asked only for a reply. That is the register for this audience and it should be the house standard, not a one off.

The methodology is the most valuable unpublished asset in the firm. Hofstede demoted from load bearing framework to background literature, because the Nigeria figures come from surveys run between

1967 and 1973 on a three country West African aggregate. HEXACO as the integrity backbone. The South African Personality Inventory for the social relational content that Western instruments miss. The GLOBE practice versus values gap as the response bias framework. Forced choice formats scored with item response theory rather than Likert scales. Nobody in this market is publishing anything like it.

The constraint you must enforce. A diagnostic on the live instrument found the emotional intelligence scale collapsed at the ceiling, the 360 funnel losing seventy two per cent of subjects at the invite step, no integrity construct present at all, and no response time captured on any item. Until those are fixed, **we do not claim predictive validity, normative percentiles or integrity measurement**, in a deck, a post or a proposal. That is a line you hold, including against enthusiasm from inside the firm.

3.5 Mezo Health

A verified specialist network and hospital operating stack, running as a CFA product with no separate registration and named client side as the firm's facility operations practice. The proposition is a private practice without building one: consult at network facilities or by teleconsultation while Mezo fills the clinic, runs the schedule and collects payment, with claims handled and advance factoring against pending claims on the growth plan.

Sell it aspirationally and never in consumer application language. Never "claim your account", never "sign up".

It has no brand guide at all. Treat it as not ready for a campaign. If Debo asks for Mezo marketing, the honest answer is that the brand definition is a piece of work that has to happen first, and it should be scoped and priced like any other brand project.

3.6 The brand hygiene backlog

These are yours to close or to price and commission. Several are cheap and have been open a long time.

- CadreHealth logomark. Highest priority.
- Maarova logomark.
- Mezo Health brand definition and guide.
- CFA SVG logo master.
- Typography locked on all three existing brand guides.
- oncadre.com registered, plus the defensive domains.
- Naming inconsistency across Med Lyfe, Medlyfe, The Lyfe Place and The Lyfe Centre in client material.
- The credential claim that appears as both sixty plus and one hundred and thirty five plus years of combined experience. Pick one, make it true, sweep every document.

Part Four: The agency: responsibilities, service levels and KPIs

This is the part Debo asked for explicitly. It sets out what the agency is accountable for, what it is not, and what it will be measured on.

4.1 What the agency owns

Owned end to end, as the accountable party.

1. **Brand.** Positioning, architecture, identity, guidelines, and the governance that keeps them applied. For our own four brands and for every client account.
2. **The editorial engine.** Everything published in the firm's name or a client's: articles, posts, case studies, decks, adverts, scripts, email.

3. **The clearance gate.** No external publication in the firm's name goes out without passing the checklist in Part Six, and you operate that gate. This is a risk control function, not a style function, and it is the single most important responsibility on this list.
4. **Digital estate.** Websites, landing pages, application surfaces the client sees, search performance, analytics and tracking.
5. **Campaigns.** Planning, production, media, sequencing, execution and reporting.
6. **Acquisition engines for our own products.** CadreHealth registrations and activations, Maarova assessment starts and organisation onboarding.
7. **The asset library.** Templates, components, reusable copy, photography, iconography, and the systems that stop us rebuilding from scratch every time.
8. **Client reporting.** Monthly for every account, internal accounts included.
9. **Capacity and utilisation.** Knowing what the team can deliver before an account is accepted.

Owned jointly, with a named counterpart.

Area	Your side	Their side
Medical accuracy of any clinical claim	Drafting and sourcing	A clinician signs off. Debo or the client's medical director.
Anything touching a live engagement or a named client	Drafting	Debo clears. Always.
Deliverability, endpoints, tracking and data capture	The requirement and the verification	Engineering builds it
Pricing of agency work	Recommendation and rate card	Debo approves
Client relationship and scope changes	Delivery and flagging	Debo or the account's sponsor owns the conversation

Never owned by the agency. The client's clinical quality. The client's tariff and pricing. Their capacity to serve the demand we create, which is the most common way a healthcare campaign fails and must be raised before a campaign launches, not after. And any claim about outcomes we cannot evidence.

4.2 The service levels we commit to

Publish these internally first, hold the team to them for a quarter on our own brands, and only then put them in a client contract.

Commitment	Standard
Acknowledge a client request	Same working day
Substantive response to a brief	Two working days
First draft of a standard deliverable	Five working days from an approved brief
Rounds of revision included	Two, then it is a change of scope
Monthly report delivered	By the fifth working day of the following month
Campaign go live after final approval	Three working days

Commitment	Standard
Emergency or reputational issue	Response within four hours, in hours

4.3 KPIs: the frame

Three levels, because three different questions are being asked. Do not mix them in one report.

Level one, agency commercial. Is this a business. Reported to Debo monthly.

Level two, account delivery. Are we doing good work for each client. Reported per account monthly.

Level three, brand and product. Are our own brands growing. Reported monthly with the internal accounts.

4.4 Level one: agency commercial KPIs

KPI	Why it matters	Proposed target, year one
Retained accounts	The base the business stands on	3 by end Q1, 6 by end Q2, 10 by year end
Monthly recurring revenue from retainers	Predictability	Rising each quarter, no quarter lower than the last
Project revenue booked	The upside, and the way new logos usually start	At least equal to retainer revenue in year one
Revenue per head	The single best measure of agency efficiency	Set a baseline in Q1 and improve it every quarter
Gross margin per account	Whether the work is priced right	No account below 40 per cent. Any account below it is re-scoped or re-priced.
Billable utilisation	Whether capacity is being used	60 to 70 per cent. Above 80 is a warning, not a win.
Logo retention	Whether the work is good	No involuntary loss of a retained account in year one
Average retainer value	Whether we are moving up market	Rising. A falling average means we are winning the wrong clients.
Proposal win rate	Whether the offer lands	Track from the first proposal. Below 25 per cent means the offer or the qualification is wrong.
Time from first conversation to signature	Sales efficiency	Track and shorten

The two numbers to watch hardest are revenue per head and gross margin per account, because they are the two that tell you whether growth is real. An agency can add clients and add headcount indefinitely while getting worse at its job, and only those two reveal it.

4.5 Level two: account delivery KPIs

Every account, ours and the clients', reports the same six.

KPI	What it tells you
On time delivery against the calendar	Whether the plan is realistic
Revisions per deliverable	Brief quality. Rising revisions almost always means the brief was thin, not that the work was bad.
Scope changes logged and priced	Whether scope is being absorbed silently, which is how agency margin dies
Client satisfaction, one question, quarterly	Early warning. Ask: would you recommend us, and what would you change.
The client's own outcome metric	Whether we are worth the fee
Hours booked against hours sold	Whether this account is profitable

The client's own outcome metric is the one that matters and it is chosen per account at the start. For a hospital, it is almost never impressions. Choose from this list and agree it in writing before any work starts.

Client type	The metric that should govern the account
Hospital or specialist practice	New patient enquiries, and enquiry to booked appointment conversion
Hospital growing a specific service line	Volume in that service line, and its share of total admissions
Practice dependent on referral	Referring clinicians active in the period, and referrals per referrer
Health plan	Enrolments, cost per enrolment, and member retention
Diagnostics or laboratory	Tests requested per referring facility
Healthtech or telemedicine	Activated users, and cost per activated user
Non governmental or public health	Behaviour completed, not reached. Screenings attended, doses given, forms submitted.

4.6 The honest attribution position

In this market attribution is imperfect and you should say so before anyone builds a target on it. People find a hospital through a relative, a doctor, a church, a WhatsApp group and a search, in that order, and often on the same day.

So instrument the one point that is reliable: ask at the point of booking. A single "how did you hear about us" field, captured at reception or in the booking flow, is worth more than every dashboard we can build, and getting it installed should be a precondition of taking a hospital account. Digital analytics then explain the shape inside the channels it can see, and we report both without pretending the first is complete.

4.7 Level three: our own brands

Brand	KPIs
Consult for Africa	Qualified inbound enquiries, insights articles published, engagement from target job titles, named references cleared, proposals requested

Brand	KPIs
CadreHealth	New registrations, activation rate of registered users, reviews submitted, salary surveys completed, readiness scores run, deliverability health, paying subscribers
Maarova	Assessments completed, organisations onboarded, coaching starts, discovery calls booked, cost per completed assessment
Mezo Health	Not yet. Gate until the brand definition exists.

CadreHealth's activation rate is the number that matters and currently it is about six per cent. Six hundred and sixty logins against ten thousand two hundred and twenty six records. Every other CadreHealth metric is downstream of moving it.

4.8 Quality gates, which are non negotiable

Four gates, applied to our own work and to every client account.

1. **Clinical accuracy.** Any clinical claim is checked by a clinician before publication. Not by us, by a doctor.
2. **Evidence.** No outcome claim, success rate, volume or comparison goes out without a source we could produce if challenged. This applies to a client's claims about themselves as much as to ours.
3. **Consent and data.** Any patient story, image or testimonial carries documented consent. Patient data is handled under the Nigeria Data Protection Act 2023, which supersedes the older regulation that most Nigerian marketing material still cites. If a client's brief asks us to use patient material without documented consent, the answer is no and it is escalated.
4. **Product claims.** Nothing may be promoted that requires regulatory registration and does not have it. This bites on any device, drug, supplement or aesthetic product a client wants marketed.

One open item to close with counsel, and it belongs on your list rather than mine. The advertising restrictions that bind registered practitioners and licensed facilities in Nigeria. We are producing marketing for doctors and hospitals who are personally subject to professional advertising rules, and we should have a written position from counsel on what a practitioner may and may not say, rather than working from what competitors appear to get away with. Until we have it, keep practitioner-facing copy factual and credential based rather than comparative or promotional.

4.9 What we will not do

Worth writing down now, because it is much harder to refuse later.

- No claim of a clinical outcome that is not evidenced.
- No before and after imagery without documented consent and without a statement of what is typical.
- No fabricated or incentivised patient reviews, on any platform, ever.
- No campaign that drives demand a client demonstrably cannot serve.
- No comparative claim against a named competitor.
- No use of a patient's identifiable image or story to sell a procedure to a vulnerable audience.
- No work for a client whose clinical quality we have reason to doubt. This one is a judgement call and it comes to Debo.

That list is also a marketing asset. In a market where the opposite behaviour is common, publishing what we refuse to do is a positioning statement that our target clients will recognise.

Part Five: Scale and execution efficiency

This is the standing ask and it does not go away when the quarter changes. The proposition is simple. **The constraint on this agency will be delivery, not demand.** Debo can bring demand. Talent can be hired. What cannot be bought late is a way of working that lets five people service twelve accounts without everything running through one person's head.

Everything in this part is work to do before the accounts arrive, not after.

5.1 Productise: standard engagement shapes

The single highest leverage thing you can do this quarter. Stop quoting bespoke and start selling a small number of known shapes with known effort, known deliverables and known price. Bespoke is available at a premium and by exception.

A starting set, to be costed against real hours on our own brands first.

Shape	What the client gets	Rough effort	Model
Brand foundation	Positioning, architecture, identity, guidelines, core messaging, applied to three surfaces	4 to 6 weeks	Project
Digital front door	Website or landing estate, search foundations, booking or enquiry capture, analytics, the "how did you hear about us" instrumentation	6 to 8 weeks	Project
Service line launch	Positioning for one service line, creative, a campaign, referrer communications, and a measurement frame	8 weeks	Project
Referral engine	Referrer mapping, communications, a referrer portal or list, and the rhythm to maintain it	8 to 12 weeks	Project then retainer
Always on marketing	Content calendar, social, email, community, monthly reporting	Monthly	Retainer
Growth retainer	Always on, plus paid media management, landing pages, testing and optimisation	Monthly	Retainer
Fractional marketing lead	A senior marketer in the client's leadership rhythm for a set number of days a month	Monthly	Retainer

Why these seven. They map to how a hospital actually buys, they sequence naturally from one another, and every one of them has already been done inside this firm at least once for an internal brand or a consulting client.

The rule that makes productisation stick: anything outside a standard shape is quoted as a change, in writing, before it is started. Absorbed scope is the most common way an agency's margin disappears and it never shows up in a report until the year is over.

5.2 The reusable asset library

Nothing should be built twice. Build the library against our own four brands this quarter, where the cost of getting it wrong is low, then apply it to paying accounts.

What goes in it.

- Templates: case study, insights article, data insight post, quote card, carousel, one page brief, campaign plan, monthly report, proposal.
- A component set for landing pages, so a new one is assembled rather than designed.
- A copy bank: approved boilerplate at three lengths per brand, approved descriptions of every service line, approved claim language with sources attached.
- A photography and iconography standard, with a solved answer to the perennial problem of finding images of African clinical settings that do not look like stock.
- Analytics and tracking as a standard install, not a per-account build.
- A campaign kit pattern. You already have one that works: the Founding Circle kit, with its channel table, owner column, per channel copy, and page edits needed before launch. Make that the template for every campaign the agency runs.

The test for whether the library is real: a new joiner should be able to produce a publishable post for any of our brands on day three without asking anyone a question.

5.3 The capacity model

You need to be able to answer, before an account is accepted, whether it can be delivered. That requires three things and none of them exist yet.

1. **A measured cost per deliverable.** Track actual hours on our own brands for six weeks. Not estimates. You will find the numbers are not what anybody assumed, and that is the point.
2. **A capacity number per person per month**, stated in deliverables rather than hours, because deliverables are what a calendar is made of.
3. **A simple model** that takes the account list and the calendar and returns whether the month is deliverable. A spreadsheet is sufficient and better than a tool nobody updates.

Plan to 70 per cent. An agency planned to full capacity has no room for the emergency, the re-brief or the illness, and those arrive every month. The slack is not waste, it is what makes the service levels in section 4.2 keepable.

5.4 One process, used for everything

The same intake, brief, production, review, publication and reporting flow for our own brands and for paying clients. Two processes means the internal work gets the sloppy one, and the sloppy one is where mistakes get made.

Intake. Every piece of work enters through one route with a one page brief: audience, the single message, the action we want, the constraints, the deadline, who clears it. No brief, no work. This alone will cut the revisions per deliverable metric more than any creative improvement.

Production. Assembled from the library wherever possible.

Review, in three passes and always in this order.

1. **Accuracy.** Are the facts right and sourced. Clinical content goes to a clinician.
2. **Compliance and clearance.** The checklist in Part Six. You own this pass and it does not delegate.
3. **Craft.** Voice, structure, design.

Doing craft first is the most common waste in agency work, because a beautifully written paragraph that fails clearance is thrown away entirely.

Publication. Scheduled, never ad hoc, with the tracking already attached.

Reporting. Monthly, same format, every account.

5.5 Tooling

Choose deliberately and keep it small. Every tool is a thing somebody has to maintain.

Need	What to look for
Work tracking	One board, one place, visible to Debo without asking
Content calendar	Shared, per brand and per account, months ahead
Asset store	One home for the library, versioned, with the current logo obvious
Social scheduling	Multi brand, with approval built in
Email	The firm sends through the transactional provider's interface. Campaign email must go through it and not through the generic mail account.
Analytics	One dashboard per account, built once from a template
Time tracking	Lightweight, and non negotiable, because without it there is no margin number and no capacity model

Time tracking will be the least popular item on this list and it is the one that makes half of Part Four possible.

5.6 The bench, and hiring order

Order	Role	Why this one next
1	Designer, in house or a reliable retained freelancer	Design is the current bottleneck and the brand hygiene backlog cannot be closed without it
2	Content and social executive	This is Ejiro. Frees you from production so you can run accounts
3	Digital and performance specialist	The moment a paid media budget arrives, this is the gap
4	Account manager	At roughly six accounts, client communication starts to eat the lead's week
5	Second writer or an editor	At the point the editorial calendar exceeds one person's output

Use freelancers deliberately, not as overflow. A tested bench of three to five people, briefed on the library and the rules, is a capacity buffer. Freelancers hired in a panic are a quality risk and they are always more expensive than they look.

Never outsource two things: the voice, and the clearance gate.

5.7 Pricing the agency

Recommend, and take it to Debo for approval. Consistent with how the firm prices everything else: anchor above the walk away, show one visible concession, never defend a price nobody has questioned.

- **A retainer floor.** Below a certain monthly fee an account costs more to service than it earns, because the fixed overhead of reporting, meetings and account management does not scale down. Calculate it from the measured hours in section 5.3 and then hold it.
- **Rate card by grade**, mirroring the consulting structure, used to build project quotes from effort rather than from a feeling.
- **Media spend passes through at cost with no commission.** This is genuinely true, it costs us nothing, it reads as unusually honest in a market where the opposite is normal, and it is a card to play late

in a negotiation rather than volunteer early.

- **Third party production passes through at cost.** Same logic.
- **A clear change of scope mechanism**, priced, in the contract.

5.8 What breaks first, and when

Know these in advance so you recognise them arriving.

At about	What breaks	Fix it before you get there
3 accounts	Nothing yet. This is the window to build the library and the process.	Use it. It closes fast.
6 accounts	Your week. Client communication and review consume the lead entirely.	An account manager, and delegated review on low risk work
6 accounts	Internal brands starve, because they have no invoice	Internal accounts run as accounts, with a protected share of capacity
10 accounts	Quality drifts, because nothing is standard	The library and the standard shapes must already exist
10 accounts	Cash, if retainers are invoiced late or media is funded by us	Never fund a client's media. Bill in advance.
12 accounts	Knowledge concentration. Only you know how things run.	Write it down as you go. This document is the start.

5.9 The quarterly efficiency question

Put one question on the agenda every quarter and answer it with evidence: **what did we build twice this quarter that we should have built once.** The answer is the next item in the asset library. Three or four quarters of that and the agency has a genuine operating advantage that a competitor cannot copy by hiring.

Part Six: The editorial engine and the clearance gate

6.1 The case study library

Ten engagements are written up in full in Ejiro's handbook, in case study form with the methodology shown. It is the firm's single largest unpublished asset and it is also the agency's credentials. Here is the index, with what you need to commission and clear each one.

#	The piece	Sensitivity	Ready to publish
1	Four evidence streams, and why the documents outvoted the interviews	**Very high.** Clinical incident material excluded absolutely.	Method only, heavily bounded
2	The ten case trace: following ten patients end to end	Medium. Principal is identifiable if described.	Yes, strongest method piece we have
3	Your feasibility study is only as good as your floor plan	Low	Yes, start here
4	Who actually decides, and why one proposal is the wrong document	Medium	Yes

#	The piece	Sensitivity	Ready to publish
5	Family hospitals and the four to four board	**Very high.** Family politics. Generic principles only.	Only fully detached from the engagement
6	Phase three, decided in phase one	Low	Yes, start here
7	Network adequacy is not network size	Low	Yes
8	Eldercare, the diaspora and the occupancy problem	Medium	Yes
9	We audited our own assessment	Internal	Not cleared. Needs Debo's decision.
10	The housemanship bottleneck	Low, public figures	Yes, as sector commentary

Start with 3, 6 and 7. They are the least sensitive, they are distinctive, and two of the three are directly relevant to the hospital owners the agency wants as clients.

Piece 9 is worth pushing for. A candid audit of our own assessment instrument would do more for Maarova's credibility with a sophisticated buyer than any number of feature posts, because the whole category rests on claims buyers cannot verify. It needs Debo's explicit decision and it is worth asking for.

6.2 The publication pipeline

Idea, drawn from the library or from live work. One page brief. Insights article first, always, because an article can be linked, sent to a prospect and indexed while a post cannot. Then the LinkedIn post that points at it. Then repurposing: a carousel, a data card, a thread, a newsletter section, a slide in the credentials deck.

One piece of work, six assets. That ratio is the efficiency measure for the editorial engine and it should be deliberate rather than accidental.

6.3 The clearance gate

You operate it. Nothing external goes out without it and it does not delegate to Ejiro until she has run it correctly alongside you for a full quarter.

- No em dash anywhere in the piece.
- No "AI powered", "AI driven" or equivalent.
- No client named, and no combination of details that identifies a client in a small market.
- No absolute client financial figure unless specifically cleared.
- No clinical incident, patient outcome, mortality or regulatory finding.
- No named individual other than our own people, and only with their agreement.
- No internal platform named inside a client deliverable.
- No price, and no defence of a price nobody has challenged.
- Every external statistic attributable to a public source, and the source recorded.
- Clinical claims signed off by a clinician.
- Consent documented for any patient story or image.
- Anything in Debo's name reads like him.
- Nothing puts a question, a brief or a concern in a client's mouth that they did not voice.

- Would the client be pleased to read this.
- Debo has seen it.

Why this sits with you rather than with him. A consultant's written document is discoverable and travels. Our engagements touch clinical incidents, regulatory exposure and family disputes, and client deliverables have been deliberately expunged of anything that could implicate a client or the firm. Marketing is the one function that takes that material and pushes it outward. The gate is the control, and a control that is only sometimes applied is not a control.

6.4 Voice governance

The rules are in Ejiro's handbook in full. As the person reviewing, these are the four failures you will actually see.

1. **Em dashes.** They reappear constantly. Check every draft.
2. **Consumer application language aimed at senior clinicians.** "Sign up", "register", "exciting opportunity", "earn extra income". Your Founding Circle copy got this right and it should be the reference every time.
3. **Copywriting where Debo's voice is needed.** He writes long connected sentences joined with because, so and which. Clipped aphorisms and "not X, but Y" constructions read as machine written even when every word is correct. When a piece is in his name, read something he actually wrote before drafting.
4. **Claims without sources.** Particularly on Maarova, where the methodology invites confident statements the current instrument does not yet support.

6.5 Cadence

Brand	Cadence
Consult for Africa	LinkedIn 4 a week, X daily, Instagram 3 a week, one insights article a fortnight
CadreHealth	X 2 to 3 daily, Instagram 5 a week, TikTok 3 a week, LinkedIn 2 a week
Maarova	LinkedIn 3 a week, email monthly plus triggered nurture
Mezo Health	Gated until the brand exists
Each client account	Per the account calendar, agreed at the start and reported against

Content pillars per brand are in the handbook.

Part Seven: Running the team

7.1 What Ejiro owns, what you own

Ejiro	You
Production against the calendar	The calendar, and what goes on it
First drafts, all brands	Final review, all brands
Social scheduling and community	Channel strategy
Asset library maintenance	Asset library design

Ejiro	You
Analytics collection	The monthly report and what it says
Campaign execution	Campaign design and the go or no go
Running the clearance checklist as a first pass	The clearance decision
Research and sourcing	Anything in Debo's name
	Client relationships, scope, pricing recommendations
	Capacity, utilisation and whether an account can be taken

Hand her real ownership early on the internal brands, where the cost of a mistake is low, and hold client accounts closer until she has run the gate correctly for a quarter.

7.2 How to review her work

Three passes in the order in section 5.4: accuracy, then clearance, then craft. Give the reason, not just the change, because a marked up draft with no reasoning teaches nothing and you will make the same correction every week.

The first four weeks will be heavy. Budget for it rather than resenting it.

7.3 Working with engineering

Several things marketing needs are built by engineering and they will not happen unless you ask specifically.

- Deliverability: bounce webhook configured and suppressions back filled before any further CadreHealth send.
- Endpoint verification before any campaign that captures data: post the real payload, read the row back, confirm the fields survived, then delete the test rows. A live page can sit in front of a dead endpoint, and a 200 response can sit in front of a parser that read nothing.
- Tracking parameters that are actually logged, rather than parameters that are merely present in a link.
- The salutation helper on any CadreHealth greeting, because the imported name fields are transposed.
- Bulk email goes through the transactional provider's interface, never through the generic mail account, which throttles batches and fails partway through.

7.4 Working with Debo

He is fast paced, wants things done rather than discussed, and prefers concise updates without trailing summaries. Bring him decisions, not options, with your recommendation first.

A monthly rhythm that will work. One page, five minutes: the commercial numbers, each account in a line, our own brands in a line, what needs his decision. Then whatever needs discussing.

Always bring him: anything naming a client, anything in his name, pricing, a new account before it is accepted, and anything you are unsure about. He would rather wave ten things through than miss one.

Part Eight: The first hundred days at function level

Days 1 to 30, instrument and stabilise

- ■ Start time tracking. Nothing in Part Four is measurable without it.

- Set the four internal brands up as accounts, each with an owner, a calendar and a notional retainer.
- Get Ejiro through the handbook and onto production on internal brands.
- Close the deliverability item with engineering. No bulk send until it is done.
- Take the KPI framework in Part Four to Debo and get the targets agreed as a set.
- Commission the CadreHealth logomark.
- Publish the first three case studies as insights articles: pieces 3, 6 and 7.

Days 31 to 60, productise

- Cost the seven standard engagement shapes against real measured hours.
- Build the first half of the asset library: templates, copy bank, campaign kit pattern.
- Write the one page brief and make it mandatory for all work, internal included.
- Draft the rate card and the retainer floor, and take both to Debo.
- Build the capacity model and use it on the current workload to establish the baseline.
- Get the counsel position on practitioner advertising onto the firm's legal list.
- Get two more named client references cleared.

Days 61 to 100, prove and prepare

- Run one full month where every account, internal and external, reports in the same format.
- Deliver the first monthly commercial pack to Debo.
- Recruit or retain a designer.
- Build the credentials deck for the agency, drawn from the published case studies.
- Take one external account through the full standard process end to end and record where it broke.
- Answer the quarterly efficiency question: what did we build twice.
- Bring Debo the agency plan he asked for: what we productise, what we charge, who we hire and in what order.

Part Nine: Decisions needed from Debo

Take these to him as a set. Each one blocks something.

1. **KPI targets.** Approve or adjust the proposed targets in Part Four, as a set rather than individually.
2. **The agency rate card and retainer floor.** Blocks every external quote.
3. **Hiring order and budget.** A designer is the immediate bottleneck.
4. **Tooling budget.** Small, and it unblocks the capacity model and the reporting.
5. **Case study 9**, the audit of our own assessment instrument. Publish or not.
6. **Named references.** Which two clients he is willing to ask, and when.
7. **Mezo Health.** Whether the brand definition is commissioned now or deferred.
8. **The Maarova claim constraint.** Confirm he backs it, so that it holds when someone inside the firm wants to claim more.
9. **Practitioner advertising position.** Whether counsel is instructed to give us one.
10. **The internal capacity share.** What proportion of the team's month is protected for our own brands once paying accounts arrive. Without a number, it will be zero.

Part Ten: Appendices

10.1 Approved boilerplate

Consult for Africa, one line.

Consult for Africa is a healthcare management consulting firm working across Africa in hospital turnaround, clinical governance, strategy and operational excellence.

Consult for Africa, short.

Consult for Africa is a healthcare management consulting firm operating from Lagos and Abuja. We turn around struggling hospitals, install clinical governance, and stand up fractional leadership for African health systems. Built by clinicians and operators who know the system from the inside, we do not hand you a report and walk away.

The agency, short.

Consult for Africa's healthcare marketing practice works with hospitals, health plans, life sciences and healthtech across Africa on brand, campaigns, digital and analytics. We have run hospitals, so we build marketing around how a hospital actually earns.

CadreHealth.

CadreHealth is the career platform for Nigerian healthcare professionals. Real salary data, honest hospital reviews, verified credentials, and a readiness score calibrated to UK, US, Canada and Gulf markets. For doctors, nurses, pharmacists and every cadre in between.

Maarova.

Maarova is a psychometric assessment and coaching platform for healthcare leaders, calibrated to the African context rather than translated into it. Six dimensions, certified African coaches, and a measurable arc from assessment to retention.

Mezo Health.

Mezo Health is a verified specialist network and hospital operating stack. Consult at our network of facilities or by teleconsultation while we fill your clinic, run your schedule and collect your payment. Your own practice, without building one.

10.2 Contact and channels of record

Web consultforafrica.com. Email hello@consultforafrica.com and partnerships@consultforafrica.com. Phone and WhatsApp +234 913 813 8553. Lagos and Abuja. LinkedIn Consult For Africa, X and Instagram @consultforafrica.

hello@ is the sender of record for everything the firm sends.

10.3 Where things live

What	Where
Ejiro's handbook, the reference layer	`docs/marketing/cfa-marketing-handbook.pdf`
This brief	`docs/marketing/cfa-marketing-operating-brief.pdf`
Brand guides	`docs/brand-guide-cfa.pdf`, `-maarova.pdf`, `-cadrehealth.pdf`
Logos	`docs/c4a-logo.png`, `docs/c4a-logo-reversed.png` for dark backgrounds, `docs/c4a-icon.png`
Social media strategy	`docs/social-media-strategy.md`

What	Where
The Founding Circle campaign kit, the template for campaign kits	`docs/maarova-circle-campaign.md`
Debo's voice reference	`docs/haven-cover-email-kabir.md`
Client deliverables	`docs/`, as script built PDFs

10.4 The three things to hold onto

One. Our proof is the method. We can show exactly how we work without naming who for, and almost no competitor can, because almost none works this way. That is the agency's credentials and the firm's authority at the same time.

Two. The constraint is delivery, not demand. Spend the quiet quarter productising, because the busy one will not give you the time and by then the cost of not having done it is permanent.

Three. You hold the gate. It is the only control between confidential client work and the open internet, and a control that is only sometimes applied is not a control.